SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab · every 6 weeks · adjuvant melanoma

Completely resected stage IIB/IIC or stage III cutaneous melanoma; the team confirms the applicable NCCP indication and timing. · NCCP 00558 · version 13 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab helps immune cells act against melanoma. After complete surgery, its aim is to lower the risk of recurrence; this does not guarantee that melanoma will not return.

Schedule
Pembrolizumab every 6 weeks. Adjuvant treatment continues for up to 12 months, subject to safety review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion reactions
- ☐ Some immune effects need long-term treatment
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment
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Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00558 v13.

SACTCheck v0.84.41
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